

CITATION: Lutes and MacPherson v. Air Canada and Jazz Air LP, 2014 ONSC 3319

ONTARIO

crew resulted in a faulty landing. I will simply quote briefly from the summary of the report of the Transportation Safety Board. It states in part:

The aircraft first contacted the runway in a left wing down side slip. The left main landing gear struck the runway first and the aircraft sustained a sharp lateral side load before bouncing. Once airborne again, the flight and ground spoilers deployed and the aircraft landed hard. Both main landing gear trunion fittings failed and the landing gear collapsed. The aircraft remained upright, supported by the landing gear struts and wheels. The aircraft slid down the runway and exited via a taxiway, where the passengers deplaned.

[2] The plaintiff, Lori Lutes, testified that she was sitting in the back of the aircraft in the 13th row. She described how the aircraft first “hit hard” and that she experienced “significant jarring”. She then described how the plane went “back up in the air”. Then there was another “significant hit and further jarring”. She said the second contact was “harder”. It was “more jarring”.

[3] At the second impact, a number of oxygen masks came down from the bulkhead. Ms. Lutes, together with other passengers, disembarked by the front door. They were taken to a nearby terminal by bus where they were tended to by a nurse. At that time, Ms. Lutes said she felt no pain.

[4] Ms. Lutes’ husband was at the airport to meet her. They retrieved her baggage and went home. Ms. Lutes said that while she was still at the airport, her first sensation of pain was a pounding headache. She was given, apparently, Tylenol, by a nurse. Then later the same day, she felt pain in her back. As a result of her continued pain, she saw her personal physician, Dr. McLaughlin, two or three days later. Dr. McLaughlin recommended massage therapy.

[5] The pain in her head and back persisted and she testified it was constant. She sought relief from massage therapy and from a chiropractor, Dr. Jennifer Heick. During the summer when she was not working, she was able to lay down. In September of 2007, she went back to her school duties. She testified the pain level increased and she would be “in tears” by the end of the day. In November of 2007 when she was back to school, she went back to Dr. McLaughlin, who recommended physiotherapy. She attended physiotherapy, at first two times and then once per week.

[6] Lori Lutes received her Masters degree in social work in 1990. She then became a counsellor with the Upper Grand District School Board in 1995. She was a social worker for the high school and for nine elementary schools. She spent 90% of her time in the schools and 5-10% of her time in travelling. She normally worked seven hours a day from 8:00 a.m – 5:00 p.m., putting in 35-45 hours per week.

[7] On the evidence, Lori Lutes was a dedicated and highly respected school counsellor. She looked forward to retiring at age 61 or 62. She was actively involved, not only in her job, but in a number of other community activities. She was on the board of the Worship Committee at her church and was involved in the choir. She was involved in youth services for Guelph and Wellington and was the treasurer of her union.

[8] She was married to her husband Douglas in October of 2002 and on the evidence, had a stable and loving marriage. Her husband, Douglas MacPherson, is an information technology specialist, working for “Staples Advantage” based at their home.

[9] Lori and her husband had interests in the theatre, in movies and in their dogs. Lori was on the Board of Directors of “World Vision” for which organization she travelled once every three years, within Canada, to the U.S. and to Mexico. She loved to travel. She described herself as a “very active person”. She attended courses at Conestoga College; pottery, knitting and quilting. She and her husband enjoyed hiking on the Bruce trail.

[10] She was a volunteer at the YMCA, leading adult fitness classes. She loved gardening.

[11] It may be noted that until November of 2010, Lori Lutes continued working full-time. In November of 2010 she went on sick leave.

[12] The evidence of Lori Lutes’ friends and co-workers describes the change in her abilities and demeanor.

[13] John Craig was a social worker with the Upper Grand District School Board. He became a colleague of Lori’s in the early 90’s. He described Lori pre-accident as “professional ... energetic”. He testified she had a “strong sense of moral and ethical values”. She was a “driven person” with a “strong Protestant work ethic”. She was always “involved in projects” and “very upbeat”. She was “incredibly professional” and a “very compassionate person ... committed and dedicated”.

[14] After the accident, John Craig testified that Lori began “questioning herself” and began to back out of committees. She would “start to wilt” and start to “stiffen up” in meetings and even to miss meetings. He sometimes had to fill in for Lori. There was “a progression over a period of time”. He testified “she was really struggling”.

[15] Barbara Witmer is a teacher of Grade 3 with the school board. She has known Lori for 18-20 years. She described Lori previously as an “energizer bunny”. She was “just always doing stuff”. She saw Lori sometimes every week. They went to movies together or out for dinner. They went to the Y, to aqua fit or walking the dog together. After the accident, the change in Lori “came on slowly”. Before the accident she was “always going”. After the accident she testified Lori would cancel out in the evening. She said that Lori doesn’t have the patience she had before, although previously she had a “really good work ethic”. That changed and now Lori “can’t sit for too long”.

[16] Diane Goodwin is as well a social worker for the Upper Grand District School Board. She has known Lori as a colleague and friend for 21 years. When Lori was in the same building she saw Lori daily. She described Lori previously as “exuberant, enthusiastic”. She was “very energetic ... fun loving”. She was further described as “playful ... fun loving ... quick to take on tasks ... a natural helper”.

[17] She noted that after the accident there was “a significant change”. It took place gradually. Ms. Goodwin testified “it’s quite obvious she is hurting”. She was “so very kind and giving ... the pain has eaten away at her competence”. Prior to the accident she was “out a lot” and a real volunteer, now Lori gets tired and she’s not out a lot. Now though Lori is trying to continue, the stress of trying to keep it going makes her sick periodically.

[18] Last year Lori didn’t go to a scheduled conference. It was an adventure based activity, from which Lori begged off. Even the drive would be too much.

[19] Vidya Narimalla is Lori's Pastor at the Kitchener Mennonite Brethren Church and has known her for a number of years, together with her husband. He testified Lori was respected as a musician at the church, playing bells. She served on the service planning committee. Pastor Narimalla described her as an "energizer bunny ... with tons of energy". She would help plan the worship services. Now she doesn't do as much. Although she loves to sing, this year she bowed out of the Christmas concert. On occasion she misses services because of pain and makes the same excuse for planning meetings. He described her as "not as loving ... not as patient".

[20] There is no question that there has been a significant change in Lori Lutes following the aircraft accident at Pearson airport. It is clear and indeed it is conceded by the defendants that she suffers from a chronic pain syndrome. It may be noted that there is little evidence of physical injury as a result of the accident. Otherwise expressed, there is no diagnosable physical cause for her current condition. It is to the credit of the defendants that they acknowledge she suffers from chronic pain syndrome, which is a genuine condition, though not medically linked to a physical injury. Perhaps the best reference I might make is the quote from Gonthier J. in the case of *Nova Scotia (Workers' Compensation Board) v. Martin* 2003 2 S.C.R. 504. At paragraph 1, Mr. Justice Gonthier states:

Chronic pain syndrome and related medical conditions have emerged in recent years as one of the most difficult problems facing Workers Compensation schemes (and I would add insurance companies) in Canada and around the world. There is no authoritative definition of chronic pain. It is, however, generally considered to be pain that persists beyond the normal healing time for the underlying injury or is disproportionate to such injury, and whose existence is not supported by objective findings at the site of the injury under current medical techniques. Despite this lack of objective findings, there is no doubt that chronic pain patients are suffering and in distress, and that the disability they experience is real. While there is at this time no clear explanation for chronic pain, recent work on the nervous system suggests that it may result from pathological changes in the nervous mechanisms that result in pain continuing and non-painful stimuli being

perceived as painful. These changes, it is believed, may be precipitated by peripheral events, such as an accident, but may persist well beyond the normal recovery time for the precipitating event. Despite this reality, since chronic pain sufferers are impaired by a condition that cannot be supported by objective findings, they have been subjected to persistent suspicions of malingering on the part of employers, compensation officials and even physicians ...

[21] A partial explanation was offered by Dr. K.S. Billing, a witness whose expert report was accepted by counsel as substantive evidence. At page 2 of his report, Dr. Billing stated:

Any patients who have pain present for more than six months do develop changes in the dorsal horn neurons as well as in the high centre of the brain. These changed neurons stimulate adjacent neurons and eventually form loops, the loops act as memory cells, memory cells keep firing impulses to the periphery and the patient perceives the pain and they have entered a state called chronic pain syndrome.

[22] There is no question that Lori Lutes suffers from chronic pain syndrome which has persisted years after the accident and the plaintiffs say may well continue for the rest of her life, or at least for the foreseeable future. It is on this basis that the claim for compensation is made.

[23] I now return to the evidence of Lori Lutes. As noted above, from November of 2010 to April of 2011, she was on sick leave. Then she went on long term disability in April of 2011 and received 60% of her salary. Her plan was to return to work full-time, but she testified she was simply not capable.

[24] During the years since the accident, Lori was subject to a plan of care over which she exerted principal control. She continued to receive chiropractic treatment from Dr. Heick and Dr. Rebecca Blackburn. She attended with Sharon Charter and Linda Ludwig, who are massage therapists. She attended with Julie Hare, a cranial sacral therapist, with Sherry Maybe and with

Ingrid Zastro, who are physiotherapists, with Anita Kieswetter, a naturopathic doctor and with Monique Peats, a counsellor.

[25] In December of 2011, a return to work agreement was reached between Lori Lutes and the Upper Grand District School Board. It was signed off by the Upper Grand Rehabilitation Consultant, the Disability Manager and the President of the Ontario Secondary School Teachers' Federation. The proposed return to work schedule involved her volunteering for three half days per week in January 2012 and then 30% paid duties (three half days per week) till the end of March 2012 and 50% paid duties (five half days per week) till the end of May 2012. From the end of May till the end of June 2012 she would be on 70% paid duties, that is two full days and three half days per week, with the expectation that after the summer vacation, as of September 1, 2012 she would return to full-time, permanent, remunerative employment. However, her chronic pain prevents her from returning to full-time employment and she currently works three and a half days per week. That situation, says the plaintiff, is likely to continue and she may not even be able to continue to work on a part-time basis until her originally planned retirement at 61 or 62.

[26] The defendants say, while not disputing her current condition of chronic pain that she is or will be in the foreseeable future capable of returning to work on a full-time basis and that therefore her claim for compensation for disability for years in the future is not justified. I therefore turn to the expert reports and the expert evidence presented at trial to assess the merit of that defence. Put quite simply, the defence is that she has failed to mitigate her damages appropriately and that therefore the claim for damages should be dismissed or reduced significantly.

The Expert Evidence

[27] Dr. Scott Garner was called to testify as to his independent medical evaluation of Lori Lutes. Dr. Garner's curriculum vitae is appended to his report, which was introduced on consent as an exhibit at trial (Tab 15, Exhibit 2). He saw the plaintiff, Ms. Lutes, on June 11, 2013. He testified at trial that she did not exhibit any "pain focused behaviour" and was of the view that she clearly suffered from a chronic pain syndrome.

[28] He testified that programs to deal with chronic pain differ. They include medication or injection treatment at "trigger points". He testified as to nerve blocks and epidural injections, but emphasized strategies of coping with the chronic pain. He said that the goal was not to "cure" pain, but to learn to cope with the pain by palliative techniques. As did other witnesses, he testified that for a soft tissue injury, such as Ms. Lutes experienced, tissue healing usually occurs within three months. If it continues, it becomes chronic. He was of the view that Lori Lutes' chronic pain was related to the faulty landing of the aircraft and he further testified that it was "not probable" that the symptoms would resolve. He would not expect an improvement in her condition and that she has lost a competitive advantage in the workplace and an impact on her personal life.

[29] Under cross-examination, he confirmed his opinion that there was no cure for her chronic pain, but that there were modalities to deal with the pain.

[30] Under cross-examination, he testified that his impression of the effects of treatment was more positive than that provided by Dr. Bowler in his evidence (which will be dealt with

presently). Dr. Garner said he could recommend injections of a less invasive sort, such as “trigger point” injections and that with such injections three to six months of relief was possible.

[31] In his report, which was accepted by the parties on consent, he stated:

In relation to the plane landing mishap of May 20, 2007, I believe that she sustained a whiplash pattern injury with involvement of the neck, shoulders and lumbar spine and secondarily, she has developed headaches.

The review of whiplash injuries noted in a review article titled “Whiplash Injury” in the Journal of Bone and Joint Surgery, 2009, 91B, pages 845-850, points out that 66% of injuries make a full recovery, which also suggests there are a significant number of people who continue to have physical limitations, and she appears to fall within this group who do not fully recover.

It is noted that recovery typically occurs in the first three months after the accident in most individuals (70% of those who will recover), and I note that she has failed to recover and thus the prognosis is not favourable for a full recovery. I believe she will continue to have limiting symptoms of pain in the neck and spinal region (with headaches) for the foreseeable future.

[32] Dr. Garner’s prognosis was stated at page 19 of his report:

Given these considerations and given my comments above, I believe she has developed a chronic pattern of symptoms which will continue for the foreseeable future, and I believe will continue to result in physical and activity limitations. In association with her symptoms of pain she has fatigue; she does not feel rested and has withdrawn from social activity.

[33] Dr. Garner then went on in his report to detail his opinion of the impact of her chronic pain on her home activities and on her employment. He concluded by saying “the above noted diagnosis, primary and secondary, appear directly related to the injuries sustained in the aviation accident of May 20, 2007”.

[34] Dealing with treatment options, he stated at page 20 of his report:

There are negative health consequences from becoming deconditioned and every effort should be made to maintain as high a level of fitness and strength as possible. She thus should continue with her gym based exercise programming to consciously counter the tendency to deconditioning ...

She notes palliative benefits which give some relief from her symptoms, and also allows her to stay as active as possible with her treatments of chiropractic therapy and massage therapy (and other manual treatments). It appears reasonable from my perspective to allow her to seek out these treatments as they are alternatives to using more medication for symptom control, and it appears probable that they are allowing her to remain more active as they use the pain barriers to a degree and thus allow her to be more active than she otherwise would be if she was not receiving these treatments. They appear to be reasonable alternatives to receiving high doses of medication.

[35] I will deal presently with evidence provided by Dr. Billing and Dr. McGoveran. Dr. McGoveran commented on Dr. Garner's report and his evidence with respect to Lori Lutes. On October 29, 2013, Dr. Garner, in a letter directed to Lori Lutes' counsel, to augment his previous report stated as follows:

With chronic pain syndromes, one must deal with probabilities and, in this case, the probability of injection treatment leading to resolution of a chronic pain problem is extremely low. Treatment often have palliative benefits and I don't deny that there may be some temporary palliative relief, but to believe that injection treatments will alleviate a chronic pain syndrome and allow her to become more functional is unlikely, in my view. She has a well-established pattern of chronic pain and the word chronic implies a long lasting pattern which is unlikely to resolve with any specific treatment. Chronic pain treatments tend to help with management and I would certainly support any provision for treatments for control of symptoms and improve the quality of her life, in fact, I have discussed the palliative benefit she received from chiropractic treatment and massage, and I have supported these as ongoing methods of giving her temporary benefit and I have also pointed out the need to work against deconditioning syndrome that comes along with being less active. I would see injection treatments as having the same type of benefit as she, herself, felt more open to the treatments she was receiving as opposed to receiving injection treatment, (which is her right) – particularly since it is very unlikely that these treatments will lead to the resolution of her pain.

[36] Dr. Kenneth Bowler provided a report as a result of his examination of Lori Lutes on May 11, 2011. Dr. Bowler is a physiatrist and a highly qualified specialist in physical medicine and rehabilitation. His qualifications are set out in his report which is found at Tab 14, Exhibit 2. Appended to his report is Appendix “B”. He provided a list of the “medical professionals” that had been involved with Lori Lutes since her accident. He began his report with a review of her medical history. At page 18 of his report, he detailed his assessment of Ms. Lutes. He stated:

She exposed herself and readily admits that she self-managed her rehabilitation care over the next 3½ years (following the accident) which she believed was directed towards minimization of discomfort and maximization of function, however, that did not work out as planned. While she maintained some reasonable sense of work continuity throughout this period of time stopping work in November 2010 in order to regroup and see whether or not with time off she could get past all of the discomfort she was experiencing, such has not been the case. Even in the absence of work, she is not experiencing longer lasting periods of relief with chiropractic care, massage therapy, naturopathic intervention, etc. All of these modalities are of limited benefit in the order of 20-30% with very brief durations less than 12-24 hours benefit ...

Within the context of my physical examination we see evidence of predominantly myofascial factors as well as mechanical factors with a normal neurological examination save mild DE sensory deficits in both lower extremities in an L4-5, L5-S1 dermatome pattern in the presence of normal musculature, normal reflexes and normal motor power. Sensory changes alone, which are subjective in nature, are less reliable than hard neurological findings, including reflex changes and/or motor power changes.

[37] Referring to her complaints (myofascial and mechanical factors) involving the neck, suboccipital region and lumbosacral region predominantly plus/minus some buttock issues, he stated that the impairments identified are physical in nature and related to the forces of the accident in question. Dr. Bowler went on to say at page 20 of his report:

She has identified that since she has engaged in the active exercise program she has derived some benefit with improved functionality, improved range of motion and this program is really in its infancy but potentially offers the best possibility

for more lasting relief patterns on a sustainable basis. This is based on evidence and literature with respect to the value of exercise in the rehabilitation of soft tissue injuries.

[38] Dr. Bowler then continued at page 21 of his report:

In the absence of any proposed treatment models, this patient will not demonstrate any further improvement in her overall subjective awareness of pain. She truly has plateaued and reached maximum therapeutic benefit from the past modalities that she has been exposed to. As such, recognizing my experience as a physiatrist and chronic pain specialist, she needs to be exposed to the following:

Dr. Bowler then went on to set out a regime of exercise and treatment to deal with her condition, which regime included facet injections (page 22).

[39] At page 23, Dr. Bowler stated:

I would also strongly endorse the exercise program she is engaged in at the present time with a trainer for 6-8 sessions that she could use over the next year. I believe that the exercise program including pool exercises, stretching exercises, bike exercises, avoiding impact loading exercises such as jogging, running, need to be maintained on an ongoing basis. To do this, she will need to accept the concept that hurt is not equivalent to harm as she exercises. She has demonstrated to herself already that she has experienced some increasing pain while exercising but, at the same time, notes the benefit of improved functionality and improved range of motion.

[40] Dr. Bowler then went on to recommend a graduated return to work. At page 24, Dr. Bowler stated:

My sense overall is that she presented in a very genuine, authentic manner without embellishment. We acknowledge that she has had problems in the past, particularly involving the low back historically and therefore is probably more susceptible to the forces of this accident over someone who has not experienced pain in times past. As such, she is more pain focused, and certainly more vigilant about trying to address her pain patterns. Unfortunately, the passive modalities that she has sought in the past 3½ years have not proven helpful and we would, therefore, suggest that she wean herself from these programs and assume a greater

locus of control with self-directed exercise programs that could be supervised and progressed over the next several months. I do believe the exercise program needs to be maintained at least for the next year or two.

Should all of our recommendations not transpire as directed, then certainly she will require some type of exposure to the chronic pain program with cognitive behavioural therapy involvement. Programs exist at McMaster with Dr. Brian Kirsch, physiatrist, and at Dr. David Corey's clinic in Mississauga. These are outpatient programs and focus on combinations of both exercise as well as cognitive behavioural therapy and trying to help patients learn to live more independently with their pain. I am hopeful, however, that the recommendations that we have made will prove helpful to her and that she will not need the services of these programs, which tend to be costly and certainly would interrupt her return to work process.

[41] Lori Lutes was referred to Dr. K.S. Billing, a specialist in the area of pain management. Dr. Billing's report may be found at Exhibit 1, Tab 4 and is also effectively reproduced in his reporting letter to Dr. Lois McLaughlin, Lori Lutes' personal physician. This reporting letter is found at Exhibit 1, Tab 2 at page 68.

[42] Dr. Billing first refers to Lori Lutes' then current medical complaints and the medications and treatments that she has undergone to date.

[43] In his impression and differential diagnosis, Dr. Billing states "Whiplash type of injury Grade II, possible cervical disc disease, myofascial pain in both shoulders, osteoarthritis in the cervical spine, atypical facial pain, TMJ disorder on both sides, cervicogenic headache and chronic pain syndrome."

[44] Dr. Billing then went on to state, as I noted above, but which quote merits repetition:

Any patients who have pain present for more than six months do develop changes in the dorsal horn neurons as well as in the high centre of the brain. These changed neurons stimulate adjacent neurons stimulate adjacent neurons and eventually form loops, the loops act as memory cells, memory cells keep firing

impulses to the periphery and the patient perceives the pain and they have entered a state called chronic pain syndrome.

Dr. Billing then continued:

I told her we can try an epidural block and epidural steroid injection and nerve blocks. I told her that her chances of getting better are 50%. In case she does improve, improvement could be temporary, could be long lasting. The pain may come back. I warned her about the possibility of epidural space infection, epidural hematoma formation, arrhythmia, convulsion, cardiac arrest, death, headache due to CSF leak, blindness, dizziness, quadriplegia, hemiplegia, total spinal anaesthetic requiring intubation and ventilation and critical care in the ICU. I also warned the patient about vasovagal syncope, and hypertension or hypotension and dizziness. I explained to the patient there is a possibility of numbness in the legs and incontinence of urine and feces which could last up to 24 hours. Complications and side effects of steroids could include: diabetes and hypertension, thinning of bones (osteoporosis), compression fractures of bones, weight gain, sometimes psychosis, adrenal gland suppression. She understood everything that I explained to her.

I gave her the information sheet which described treatment offered in my Pain Clinic, outcome of the treatment, side-effects and complications. She decided to go home and think about it. I told her to only come back if she would like help otherwise she does not need to make any more appointments.

[45] When she testified, Dr. McLaughlin said she did discuss the risks of the “Billing treatment” with Lori.

[46] Given the qualified chances of success with Dr. Billing’s treatment and the risks involved, Lori Lutes, apparently after discussion with her husband, decided not to proceed with intervention by Dr. Billing. Dr. McLaughlin testified she “didn’t disagree with Lori’s decision”.

[47] Dr. Lois McLaughlin has been Lori Lutes’ personal physician throughout the course of her treatment. Although she has seen the plaintiff with some frequency, particularly in more recent times, she has not on the whole taken an active role in Lori Lutes’ treatment regime. This has been effectively supervised by Lori Lutes herself. Dr. McLaughlin testified that Ms. Lutes

was “very compliant with treatment”, “she was trying to stay stable”. She further testified that she did not believe that an MRI or CT scan would be appropriate because the pain was myofascial in origin. She said that she would not refer Lori to a pain clinic.

[48] Dr. Bruce McGoveran was called as an expert witness by the defendants. He is a specialist in occupational medicine and his qualifications are impressive, as were the qualifications of the other medical experts who testified or submitted reports. His curriculum vitae is found at Tab 3 and Tab 4 of Exhibit 15. He testified at trial and his report (filed on consent) is found at Tab 1 of Exhibit 15.

[49] Dr. McGoveran met with Lori Lutes on the 13th of July 2012. He had reviewed much of the history to that date, together with the report of the Transportation Safety Board with respect to the relevant incident.

[50] When Dr. McGoveran testified, he stated that he was “surprised of the decision to stop working” in November of 2010. He stated that “ceasing work in its entirety is a serious thing”. It is a “last resort” in the management of any injury. He further stated that any individual “benefits from being at work” and that only when no alternative is available should one be deprived of work. He noted that on the material before him that there was “no evidence to suggest her performance was impaired” and that there was no evidence of physical injury, no evidence of organic pathology. There was “no medical contraindication to justify termination”. It was clear in his opinion, shared by other witnesses in the case, that Ms. Lutes’ pain was not associated with any pathologic damage.

[51] Dr. McGoveran went on to say that he was surprised that Lori Lutes had not accepted Dr. Billing's recommendation with respect to injections. In his opinion "injections are a safe and viable option".

[52] He went on to note that her actual employment was not "physically demanding" and repeated that "removing someone from the workplace is a measure of last resort". He testified that in his view there were alternatives to actually quitting and that there were treatment options to her pain that had not been explored. He noted there had been no cognitive testing, while confirming that Lori Lutes might actually be in pain.

[53] In his view, returning again to the treatment offered by Dr. Billing, he noted a 50% chance of improvement in her symptoms, with in his view a very low risk. Dr. McGoveran disagreed with Dr. McLaughlin's opinion that Lori Lutes was correct in rejecting Dr. Billing's treatment. In his view a comprehensive pain management program, such as that offered by Mount Sinai was long overdue.

[54] In his report, Dr. McGoveran notes Dr. McLaughlin's letter of the 4th of November 2010 advising that Lori Lutes not work for 14-16 weeks. He added "however I see no substantive discussion about how the decision to stop work clearly an important one arose". He also noted the Upper Grand District School Board's medical certificate Dr. McLaughlin completed on the 18th of November 2010, in which she advises that Ms. Lutes cannot work. Dr. McLaughlin notes only mild competency limitations and pain mediated deficiencies in various cognitive areas. Again, he could see no commentary indicating how Dr. McLaughlin came to her conclusions.

[55] Dr. McGoveran went on to note a letter dated the 8th of February 2011 from Lori Lutes' chiropractor, Dr. Heick, advising that Ms. Lutes would then require a long term leave to allow her body a break from the stresses and strains of her job. Dr. McGoveran stated "I see no other documentation in Dr. Heick's notes as to how she came to this conclusion". Dr. McGoveran went on to note that there had been no expressions of concern with respect to Lori Lutes' performance in her employment.

[56] With respect to the source of her problems, Dr. McGoveran stated at page 15 of his report:

The most probable explanation for Mrs. Lutes' symptoms is that they constitute the sequeli of her body's exposure to the energy of the aircraft impact with the runway. The resulting soft tissue pain she reports is an elegist to that reported by people involved in motor vehicle accidents. These symptoms are generally encapsulated under such terms as myofascial strain and mechanical neck or back strain.

[57] Dr. McGoveran went on to state in his report:

In general, soft tissue injuries of the kind Ms. Lutes sustained resolve over a course of days to weeks and, at most, months. Indeed, as I noted in my description of Dr. Heick's clinical records (see history of present health concerns), Ms. Lutes' recovery initially appeared in accordance with this timeline. I refer you again to the progress reports of 26 October 2007 and 12 January 2008, respectively. I acknowledge from the content of these reports that it appears Ms. Lutes' rate of recovery is slowing as of January 2008. This is not, though, surprising, for it is often the case of the final phases of a recovery take the longest period. I remain, unable however, to understand why Ms. Lutes' recovery appears to have stalled and remain incomplete thereafter, and I see no documentation from any treating health professional to explain the persistence of Ms. Lutes' symptoms far beyond their expected duration.

[58] In his report, Dr. McGoveran continued to state:

Ms. Lutes has not pursued any of the treatment options Dr. Billing offered in January 2012. For her part, Ms. Lutes tells me her reluctance to proceed with any of these recommendations relates to the range of complications Dr. Billing described. To be sure, Dr. Billing mentions a lengthy list of potential complications, many serious, in his letter of 6 January 2012. Apart from this letter, though, are the probabilities of occurrence of any of these serious complications. Most procedures, even those involving injections and which are considered by their nature to be minimally invasive, carry risk. However, the magnitude of the risk, especially for a serious complication, is quite low. I suggest it might be helpful to characterize these risks with reference to the risk of other activities in which Ms. Lutes has engaged since 20 May 2007. I note, for example, that Ms. Lutes has flown since 20 May 2007. Since this is a risk level she is comfortable assuming, it might be helpful to compare the risk of receiving injections of the kind Dr. Billing offers with the risk of flying, to the extent the two are comparable. On reflection, I regret not asking Ms. Lutes about the measures she took to clarify this matter with Dr. Billing, for such knowledge is clearly essential in coming to an informed opinion about the value of such treatment ...

I also note that other recommendations Dr. Bowler makes in his report of 11 May 2011 remain untried. For example, Dr. Bowler notes the failure of the passive treatment modalities in which Ms. Lutes participated over the past approximately 3.5 years (as of the timing of Dr. Bowler's assessment), and he suggests that Mr. Lutes wean herself from them. Dr. McLaughlin's EMR entries of 2012, however, confirm that Ms. Lutes was still using them. And based on the symptom levels Ms. Lutes reports to Dr. McLaughlin in early 2012, these modalities were no more effective than in years past. It also appears that Dr. Bowler makes further recommendations, for example for Ms. Lutes to attend a chronic pain program with cognitive behaviour therapy. However, as Dr. Bowler intended this option for the possibility of Ms. Lutes attempting and not deriving adequate benefit from an exercise program and, presumably, some form of injection based pain management, which has not occurred, I will defer comment on this point ...

In my experience people reporting pain of the intrusiveness Ms. Lutes describes will attempt any reasonable treatment plan sanctified by a physician. And in my opinion, the options put forward by Dr. Billing qualify such a plan. Moreover, it is generally accepted within the healthcare community that when conservative treatment is not having the desired effect, it is appropriate to consider more invasive surgery. And until Ms. Lutes has attempted them, we cannot know the true impact of her symptoms on her level of function. Ensuring all reasonable treatment options have been exhausted is particularly important in this instance because of the inherently subjective nature of the symptoms, pain, which Ms. Lutes (and several of her healthcare professionals) cite as the reason for her disability. Clearly, if any of Dr. Billing's treatments can even partially alleviate Ms. Lutes' pain, her level of function ought to improve.

[59] Dr. McGoveran then went on to state:

Nevertheless, given the information available to me as I write this report, I see no medical contraindication to Ms. Lutes working full-time in her current role as a social worker for the UGDSB. My opinion follows from several factors. First, there is the sedentary nature of Ms. Lutes' work. From my review of this work, I detect no work place factors liable to provoke market worsening of her headaches, neck pain, or low back pain. This is not work, for example, that requires heavy lifting and awkward postures, or repetitive neck movements, which are often associated with increased reports of neck and back pain, respectively. I acknowledge that Ms. Lutes' work has the potential for exposure to prolong static postures, and I acknowledge the potential for such postures to provoke musculoskeletal pain. One can readily mitigate these pains however, with simple strategies that include periodic repositioning and what are frequently called micro breaks, or transient pauses to stretch, walk about or engage in other means of breaking the static postures. I see no reason why Ms. Lutes cannot adopt these same strategies with the same success others generally report ... I see no medical reason why Ms. Lutes cannot sustain regular, predictable, full-time employment in her current role.

While on the subject of Ms. Lutes' fitness to work, I must also express concern with the decision of her various healthcare providers to support a medical leave, first of 14-16 weeks, and then a total of approximately 14 months. In none of the providers documentation contained in the brief is there a clear, cogent rationale for either leave.

[60] Dr. McGoveran then returned to the recommendations provided by Dr. Billing. He stated:

I renew my call for Ms. Lutes to clarify the likelihood of a serious complication were she to undergo any of the treatments Dr. Billing recommends. Ms. Lutes and I also discussed the option of obtaining a second opinion as regards injection treatment for her various pains. I endorse this idea if the outcome of her clarifying complication rates with Dr. Billing are not to Ms. Lutes' satisfaction.

[61] Dr. McGoveran then continued:

The medical brief does contain letters from Dr. Heick, Ms. Deanna Brenneman (osteopathy), and Ms. Linda Ludwig (massage therapy) for the February-March 2011 period, all advising of the need for Ms. Lutes to remain off work on a longer term basis ...

[62] Dr. McGoveran then expressed his reservations about the opinions that it was necessary for Ms. Lutes be on an extended leave of absence.

[63] By letter dated 17 October 2013 directed to counsel for the defendants, Dr. McGoveran again expressed his concern why Lori Lutes had not further considered the injection treatment offered by Dr. Billing.

[64] These then are the opinions expressed by the medical experts who have offered their opinions to assist the court. It is not for this court to pretend a medical expertise. Should Lori Lutes have taken advantage of Dr. Billing's recommendation for injection therapy, would the results have assisted her in dealing with her chronic pain symptoms? These are questions which are at least as yet unanswered. Given the chances of success (perhaps 50%) and the possible risks, Lori Lutes cannot be criticized for declining this treatment. However, her decision to decline the treatment, in my view, must be considered in my determination of her efforts to mitigate her damages..

[65] As well, there are other treatment options that have been recommended to Lori Lutes, which she has not fully explored, which I take into consideration.

[66] While Dr. Lois McLaughlin has continued to serve as her personal physician, Lori Lutes has been, it would appear entirely, the supervisor of her healthcare providers. Very much to her credit, she has explored and availed herself of many options. They include massage therapists, chiropractors, physiotherapists and an osteopath, a counsellor and a naturopathic doctor. I am sure the many people listed in the material provided to the court, some of whom have provided reporting letters or have testified, are dedicated healthcare providers. However, while the relief

they provided may have been of some significance, it has all been temporary. Lori Lutes still suffers from her chronic pain syndrome which impacts significantly on her personal life and on her professional life as well.

[67] It may be noted that more than one of her healthcare providers and expert witnesses have recommended an exercise regime with a personal trainer. Lori Lutes has never taken advantage of a personal trainer. It may be that because she has been involved in various forms of exercise to keep fit for many years, she feels she does not need a personal trainer. Yet I cannot help but consider that an experienced, qualified personal trainer to supervise her exercise regime for an extended period would be of significant help to her in controlling, if not resolving, her symptoms.

[68] I therefore feel that in failing to follow the recommendations of many involved in the treatment of her symptoms, that she involve a personal trainer and exercise regime is a failure to responsibly mitigate damages on her part. She has for years been effectively the only person in charge of her rehabilitation. She has availed herself of many caregivers, a number of whom have recommended a personal trainer to assist in her rehabilitation. She has failed to take advantage of this wise advice.

[69] I also conclude that her failure to take advantage of the advice of Dr. Billing with respect to an epidural block, epidural steroid injection and nerve blocks amount to a failure to properly mitigate her damages on her part. Dr. Billing's report states that her chances of getting better are 50%. That in fact constitutes a reasonable likelihood, even with Dr. Billing's qualifications that improvement could be temporary or it could be long lasting. Given the significant efforts that Lori Lutes has already taken to deal with her chronic pain, I do not understand why she would

not try this approach. Though Dr. Billing as a cautious and prudent physician, outlined to her in detail the potential risks of injection therapy, given the other evidence in the case, including that of Dr. Bowler and Dr. Garner, I conclude that the risks are relatively minor as compared with the potential benefits of such treatment. In sum, she cannot claim compensation for her chronic pain for years in the future or for the rest of her life, if there is a real probability that injection therapy, as proposed by Dr. Billing, would alleviate her chronic pain syndrome. I conclude that real probability does exist. Thus any award of damages must be reduced by her failure to mitigate her damages.

[70] The onus to establish a failure to mitigate damages is clearly on the defendants. I conclude that such onus has been satisfied on a preponderance of the evidence at trial. Particularly based upon the expert evidence before the court, I conclude on a preponderance of such evidence that the plaintiff has failed to avail herself of treatment modalities and a treatment regime which have a reasonable prospect of improving her condition and reducing her claim for damages in the future. There is no evidence of the possibility (let alone the probability) of a recovery “overnight”. However, I must consider the reasonable probability of a change in her condition, and her decision to not avail herself of any and all treatment modalities that might reasonably lead to a positive change in her condition, in assessing her entitlement to compensation.

The Assessment of Compensation

[71] I have no quarrel with the evidence offered by Ms. Deborah Carter, who testified as an expert witness in the area of labour market economy. Her calculations were, however, based upon the premise that Lori Lutes would be unable to resume her professional duties in the

foreseeable future or indeed to her originally planned retirement date. Her calculations, therefore, must be considered in the context of what I conclude is Lori Lutes' failure to date to properly mitigate her damages and as well to take into account other future contingencies such as Lori Lutes' ability to return to full-time employment.

[72] I am mindful of the considerable jurisprudence cited by counsel for the plaintiff and for the defendants. I am particularly mindful of the decision of the Ontario Court of Appeal in the case of *Schrump et al v. Koot et al* (1977), 18 O.R. (2d) 337. In that case, Mr. Justice Lacourciere stated:

In assessing damages for personal injuries the award may cover not only all injuries actually suffered and disabilities proved as of the date of trial, but also the "risk" or "likelihood" of future developments attributable to such injuries. It is not the law that a plaintiff must prove on a balance of probabilities the probability of future damage; he may be compensated if he proves in accordance with the degree of proof required in civil matters that there is a possibility or a danger of some adverse future development.

The Court held that the trial Judge was in error in awarding nothing for the future disability of the plaintiff where the medical evidence had placed the possibility of disabling arthritis at somewhere between 33 and 50%. Robertson, J.A., said at p. 346 D.L.R., p. 564 W.W.R.:

In my respectful opinion, the learned Judge misdirected himself. It is a fundamental rule that in civil cases questions of fact are to be decided on a balance of probabilities; this a matter of proof. But it is not equally true that damages in respect of things which have not yet developed may only be awarded if it is probable that they will develop and may not be awarded if it is only possible that they will develop. One can decide on a balance of probabilities that something in the future is a possibility, and in appropriate circumstances that possibility can be taken into account in assessing damages; in such a case it is not essential, before damages can be assessed for the thing, to decide on a balance of probabilities that the thing in future is a probability. When the word "probability" is used in such a context there is an inclination to contrast it with the word "possibility". That can be avoided by using instead the word "risk", or perhaps "danger" or perhaps "likelihood". Then one can say, without the danger of being misunderstood, that one can decide on a balance of probabilities that there is a risk of

something happening in the future. In an appropriate case such a risk can be taken into account in assessing damages for the wrongful act or default that caused it. As put by Cartwright, J., on behalf of himself and Rand, Kellock and Fauteux, JJ., in *Archibald et al. v. Nesting et al. and Dalton*, [1954] 1 D.L.R. 347 at p. 361, [1953] 2 S.C.R. 423:

“... the innocent person who has been gravely injured by the fault of another should not be called upon to bear all the risk of the uncertainties of the future.

The degree or risk, danger or likelihood in each case will, of course, be taken into account in assessing damages as a matter aggravating the injury. When “probability” and “possibility” are used, which term fits the circumstances cannot be decided by any mathematical proposition.

[73] In the leading case of *Janiak v. Ippolito*, [1985] 1 S.C.R. 146, the Supreme Court of Canada had occasion to deal with the issue of mitigation in a case where the recommended surgical treatment entailed a 70% chance of success and if successful 100% chance of recovery with then the possibility of the plaintiff’s returning to work. The Supreme Court determined that the question of whether or not a person has been reasonable in refusing to accept medical treatment is for the trier of fact to decide. In making that finding, the trier of fact must take into consideration the degree of risk from the surgery, the gravity of the consequences of refusing it and the potential benefits to be derived from it.

[74] Madam Justice Wilson stated:

In making his finding as to the reasonableness or otherwise of a refusal of medical treatment, the trier of fact will also, of course, take into consideration the degree of risk to the plaintiff from the surgery (*Taylor v. Addems and Addems*, [1932] 1 W.W.R. 505 (Sask. C.A.)), the gravity of the consequences of refusing it (*Masney v. Carter-Hall-Aldinger Co.*, [1929] 3 W.W.R. 741 (Sask. K.B.)), and the potential benefits to be derived from it (*Matters v. Baker and Fawcett*, [1951] S.A.S.R. 91 (S.C.)).

While a plaintiff has the burden of proving both the fact that he has suffered damage and the quantum of that damage, the burden of proof moves to the

defendant if he alleges that the plaintiff could have and should have mitigated his loss. That this is the law in Canada has been clearly stated by this Court in *Red Deer College v. Michaels*, [1976] 2 S.C.R. 324, and more recently reaffirmed by Estey J. in the *Asemara Oil* case, *supra*. In *Red Deer Laskin* C.J. said, at p. 331:

If it is the defendant's position that the plaintiff could reasonably have avoided some part of the loss claimed, it is for the defendant to carry the burden of that issue, subject to the defendant being content to allow the matter to be disposed of on the trial judge's assessment of the plaintiff's evidence on avoidable consequences.

[75] In this case, the defendants do not contest liability. What is in issue is the quantum of damages. It is conceded by the defendants that the plaintiff suffers from a chronic pain syndrome, which has significantly impacted upon her quality of life and at least to some degree has impacted on her ability to earn an income. The defendants contend, however, that the plaintiff has failed to properly mitigate the impact of her chronic pain condition, upon both her personal and her professional life. As the result thereof, the defendants plead that the court should limit the compensation which is claimed by the plaintiff. To some extent I agree.

[76] In effect, the defendants also plead the uncertainty of the plaintiff's claim in the future. They point out that her chronic pain was not functionally disabling in the past. She continued to work in a full-time capacity until November 2010, some 3½ years from the date of the incident and has continued to work periodically, at least part-time, up to the present. The plaintiff points out that within four months of the aircraft incident, she travelled to China for two weeks and then in August of 2010, four months prior to taking her extended sick leave, she travelled to Cambodia. There is every expectation, say the defendants, that sometime in the future, certainly well before her normal retirement date, she will be able to return to work.

[77] I agree. Lori Lutes has always been an exceptional person. She has gained the affection and respect of her fellow workers and those involved with her socially. That she suffers from chronic pain, which has impacted on both her social and professional life, is not in dispute. However, I would hope and expect, particularly if she takes advantage of other treatment offered by healthcare professionals, she will be once again functioning at a level that she did prior to the accident at Pearson airport.

[78] Her failure to try other options to mitigate and the contingency for improvement under any circumstances in the future should in my view have some impact upon her losses to date and in the future. Taking all these factors into account, I assess her general damages at \$110,000. Her past income loss, I assess at \$168,000. Taking into account, once again her failure to mitigate and future contingencies, I assess her future income loss and loss of value in her pension at \$600,000. For future care, I award the amount of \$60,000. For special damages to date, I award the amount of \$46,496.75. For the O.H.I.P. claim, I award, as requested, \$3,000. For the family law claim, I award the amount of \$25,000.

[79] A total award for damages is therefore \$1,012,496.75. If counsel are unable to agree on costs, they may address me in brief written submissions, directed to me in chambers, within 60 days of publication of this judgment.

[80] I wish to thank counsel for their considerable assistance and commend them on their thorough preparation and advocacy in this most challenging case.

R.D. Reilly J.

Released: May 14, 2014

CITATION: Lutes and MacPherson v. Air Canada and Jazz Air LP, 2014 ONSC 3319

COURT FILE NO.: C-406-09

DATE: 2014-05-14

ONTARIO

SUPERIOR COURT OF JUSTICE

Lori Lutes and Robert Douglas MacPherson

Plaintiffs

– and –

Air Canada and Jazz Air LP, as represented by
its general partner, Jazz Air Holdings GP Inc.,
carrying on business as Air Canada Jazz

Defendants

REASONS FOR JUDGMENT

R.D. Reilly J.

Released: May 14, 2014

lr